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METRO PULMONOLOGY GROUP

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REFERRAL LETTER

**REFERRAL TO MEDICAL ONCOLOGY FOR
CHEMOTHERAPY/IMMUNOTHERAPY EVALUATION**

DATE OF REFERRAL: 2026-02-15

TO: Dr. Sandra DemoOncologist, MD
Medical Oncology
Regional Oncology Institute
800 Cancer Center Boulevard
Riverside, ST 00000

FROM: Dr. Anthony SampleDoc, MD
Pulmonology
Metro Pulmonology Group
2100 Respiratory Drive, Suite 300
Riverside, ST 00000
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RE: PATIENT REFERRAL

Patient Name: David MockPatient
MRN: TEST-010
Date of Birth: 1952-10-05
Sex: Male
Age: 73

REASON FOR REFERRAL

Newly diagnosed non-small cell lung cancer (adenocarcinoma) with mediastinal lymph node involvement. Referral for systemic therapy evaluation including immunotherapy given PD-L1 expression.

CLINICAL SUMMARY

Mr. MockPatient is a 73-year-old male, former smoker (45 pack-year history, quit 8 years ago), who presented with a 3-month history of persistent cough and unintentional 10-pound weight loss. Workup revealed a right upper lobe lung mass with mediastinal lymphadenopathy.

KEY DIAGNOSTIC WORKUP:

- Chest CT (2026-01-20): 3.8 cm right upper lobe spiculated mass with enlarged right paratracheal (station 4R, 2.1 cm) and subcarinal (station 7, 1.6 cm) lymph nodes.
- PET-CT (2026-01-28): FDG-avid right upper lobe mass (SUV max 9.2) with FDG-avid mediastinal lymph nodes (stations 4R, 7). No distant metastases. Brain MRI negative for metastatic disease.
- CT-guided core needle biopsy (2026-02-05): Moderately differentiated adenocarcinoma. TTF-1 positive, Napsin A positive.
- EBUS-TBNA (2026-02-10): Station 4R lymph node positive for metastatic adenocarcinoma. Station 7 lymph node positive for metastatic adenocarcinoma.

STAGING:

Clinical Stage IIIA (cT2a N2 M0, AJCC 8th Edition)

MOLECULAR AND BIOMARKER RESULTS:

- **PD-L1 (22C3): TPS 55%**
- EGFR: Wild type (no activating mutations)
- ALK: Negative (IHC and FISH)
- ROS1: Negative (IHC)
- KRAS G12C: Negative
- BRAF V600E: Negative
- MET exon 14 skipping: Negative
- NTRK fusions: Negative

PULMONARY FUNCTION:

- FEV1: 2.1 L (68% predicted)
- DLCO: 58% predicted
- Adequate pulmonary reserve for concurrent chemoradiation if indicated

RELEVANT MEDICAL HISTORY

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- COPD (moderate, GOLD Stage II), on tiotropium and albuterol PRN
 - Hypertension, controlled on amlodipine 5 mg
 - Type 2 diabetes, controlled on metformin 1000 mg BID (HbA1c 7.1%)
 - Coronary artery disease, s/p PCI with DES (2019), on aspirin and atorvastatin
 - ECOG Performance Status: 1

MEDICATIONS

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1. Tiotropium 18 mcg inhaler daily
 2. Albuterol HFA 90 mcg PRN
 3. Amlodipine 5 mg daily
 4. Metformin 1000 mg BID
 5. Aspirin 81 mg daily
 6. Atorvastatin 40 mg daily
 7. Omeprazole 20 mg daily

ALLERGIES: Sulfa drugs (rash)

DISCUSSION

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- Given stage IIIA (N2) disease with PD-L1 TPS 55% and no targetable driver mutations, this patient may be a candidate for:
1. Concurrent chemoradiation (definitive) followed by durvalumab consolidation (PACIFIC regimen)
 2. Chemoimmunotherapy combination (if radiation not feasible given PFTs)

The patient's cardiac history and moderate COPD should be considered in treatment planning. He maintains adequate performance status (ECOG 1) and has expressed willingness to undergo systemic therapy.

I would appreciate your evaluation and treatment recommendations. Please coordinate with radiation oncology as well for the concurrent approach if deemed appropriate.

Thank you for your consultation on this patient.

Sincerely,

Dr. Anthony SampleDoc, MD
Board Certified Pulmonology and Critical Care Medicine
Metro Pulmonology Group
Date: 2026-02-15